Acceptability, Appropriateness, and Feasibility of a Training Module Targeting Clinician's Ability to Respond to Empathic Opportunities in Suicidal Patients - Pilot Study

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Background

Empathy has long been hypothesized as a key element of the change process in psychotherapy. It can be clinically defined as understanding what another person is experiencing, or trying to express. Empirical literature has indicated the positive association of empathy with treatment outcomes in numerous clinical settings. While empathy is understood to be a core component of a healthy therapeutic alliance, the optimal ways to increase it require further exploration, especially in regard to clinical encounters with high risk patients. The goal of this study is to develop and assess the appropriateness, feasibility, and acceptability of a new training module based on the Empathic Communication Coding System framework, a validated observer-based measure of clinicians' responses to empathic opportunities in therapeutic interactions. This module provides a didactic training for clinicians to help them develop effective verbal empathy skills when interacting with patients in general, and high-risk patients in particular.

Research Question and Hypothesis

1. Do participants find the Verbal Empathy Training Module appropriate, feasible, and acceptable to incorporate in their training working with patients, especially those who are considered high-risk?

   H1: We hypothesize high perceived acceptability, appropriateness, and feasibility, indicating that the module is understandable, clinically relevant, and practical for training clinicians to increase their usage of verbal empathy during patient interactions.

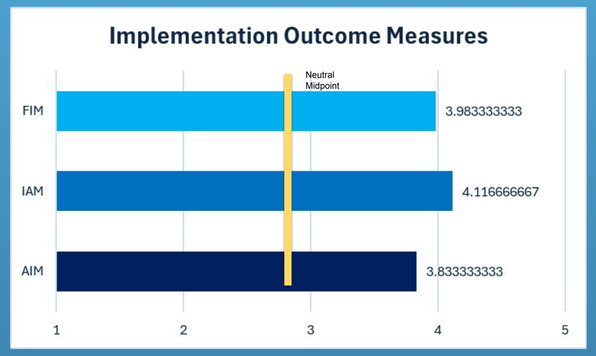
2. Do participants find the Verbal Empathy Training Module as an effective way to teach clinicians how to increase their verbal empathy?

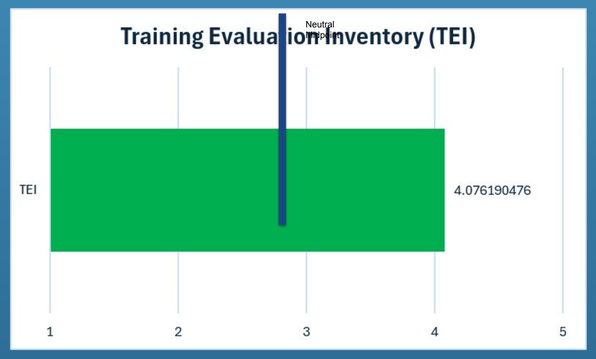
   H2: We hypothesize that participants will exhibit high perceived effectiveness for this training, and regard it as a suitable way of increasing verbal empathy in clinicians.

Method

- N=15 individuals with a graduate degree or higher in psychology or a related field participated in the pilot
- Participants completed the demographic survey, the module, and post-module implementation and perceived effectiveness measures in Qualtrics
- Acceptability, appropriateness, and feasibility were assessed using modified versions of the Acceptability of Intervention Measure (AIM), Intervention Appropriateness Measure (IAM), and Feasibility of Intervention Measure (FIM),
- Perceived effectiveness of training was evaluated using the Training Evaluation Inventory (TEI)
- Participants were encouraged to offer their opinions of the module, and suggestions for its improvement in open text boxes

Results





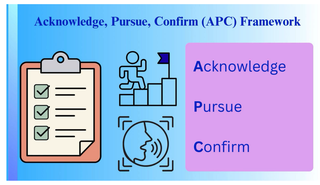
Feedback

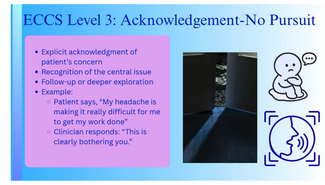
- The module appropriately highlighted the importance of empathy.
- General guidelines is not the way to go, and patients should be responded to on a case-by-case basis.
- Helpful for clinicians for whom this style of interacting with patients is not common sense.
- Visuals were calming, and relevant to the material being delivered.
- The levels of the ECCS module were explained well, and went beyond simply highlighting the importance of just empathy.
- The module did a good job of showing that empathy had discrete "borders" to it.
- Module was engaging and easy to follow.
- More examples would have been helpful in order to better understand the material.
- The module should address how patients may react to realizing that clinicians are reacting to empathic opportunities in predetermined ways. It would be a good idea to devote time to discussing other approaches clinicians can rely on when they need to demonstrate empathy towards a patient.

Suggestions for Improvement

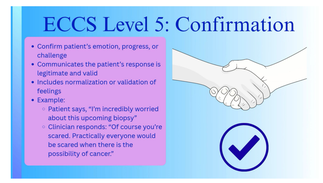
- The content of the module would be better received if it was delivered by real people with a demonstration.
- The module should discuss how different patients may require clinicians to employ empathy in different ways.
- Length should be cut down.
- A slide at the end that summarizes all the different levels of the ECCS would be useful.
- Greater amounts of time should be devoted to ECCS levels 0-2.
- Examples should follow a single patient with the same issue rather than showcasing a range of different issues.
- Devote time to explaining how using empathy in a session will result in benefits to a patient over the long-run.

Selected Slides

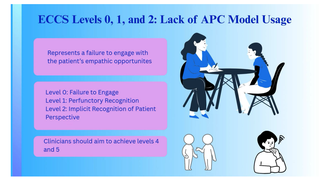












Discussions

The data indicate that the training module was perceived favorably across acceptability (M = 3.833) appropriateness (M=4.117), feasibility (3.983), and perceived effectiveness (4.076), with all scores being above the neutral midpoint of 3.

Feedback was generally positive, with special praise being given to the module's visuals, the information provided on empathy, and the level of engagement achieved.

Participants suggested 1) adding a greater level of depth to the examples, 2) adding a summary slide at the end, 3) changes to the structure of the presentation (ex. In-person instructors, answerable questions throughout the module), and 4) including information about how empathy may need to be deployed in different manners for separate patients.

Limitations & Future Directions

Sample size (N=15) was limited, as this was only a pilot study. Participants were majority female (73%) and 73% possessed a masters degree as their highest level of education.

Future studies should first and foremost attempt to have a much larger and diverse sample. The module additionally should be changed in order to accomodate the provided feedback. Namely, this would be greater emphasis on how patients will respond to the clinician demonstrating higher levels of empathy to them, and more concrete examples.

References

- Ritzmann, S., Hagemann, V., & Kluge, A. (2014). The Training Evaluation Inventory (TEI)—Evaluation of training design and measurement of training outcomes for predicting training success. *Vocations and Learning, 7*(1), 41–73. https://doi.org/10.1007/s12186-013-9106-4
- Weiner, B. J., Lewis, C. C., Stanick, C., Powell, B. J., Dorsey, C. N., Clary, A. S., Boynton, M. H., & Halko, H. (2017). Psychometric assessment of three newly developed implementation outcome measures. *Implementation Science, 12*(1), Article 108. https://doi.org/10.1186/s13012-017-0635-3